True North Systems

True North Continuity — Distributed Intrusion-Safe Healthcare Records

Marcus Rydberg · Houston / Sweden · 2026

THE PROBLEM

Healthcare's Ransomware Crisis

Most expensive industry for data breaches — fourteen consecutive years. Change Healthcare alone: $2.4B in costs, 192.7 million individuals affected. 293 ransomware attacks on healthcare in nine months, up 30% year over year.

$9.77M	1,542	293
Avg breach cost IBM 2025	Confirmed breaches DBIR 2025	Ransomware attacks 9 months, +30% YoY

THE SHIFT

Attackers Aren't Just Locking Data — They're Stealing It

Extortion-only attacks rose from 4% to 12% of healthcare ransomware in three years.
When backup data is exfiltrated, every existing solution fails.

CORE THESIS

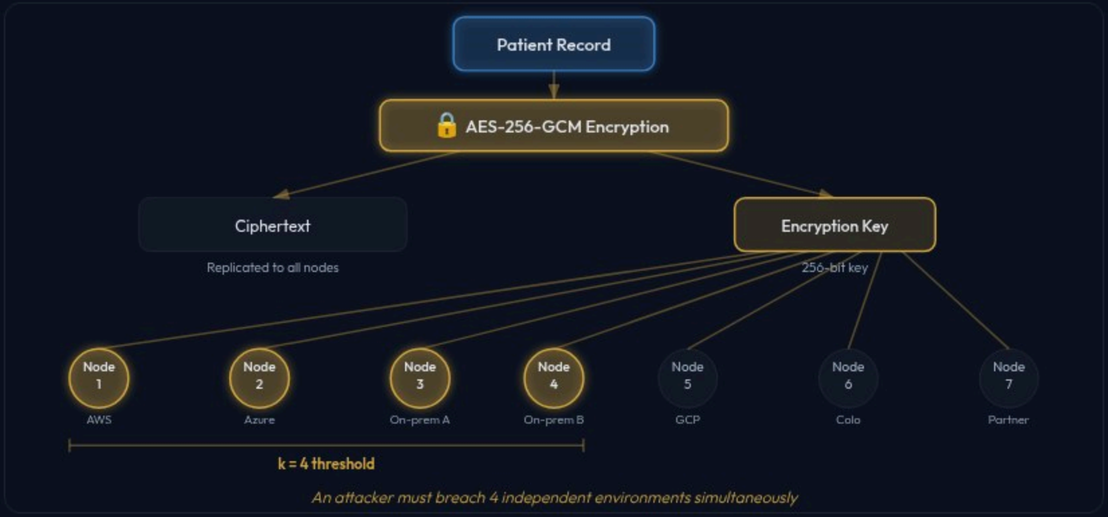
> **Recovery is solved. Exfiltration is not.**

Immutable backups protect against deletion. No one protects against theft.

THE SOLUTION

Intrusion-Safe Healthcare Archive

Continuous ADT capture, AES-256 encryption, Shamir key-splitting across independent nodes.



> *A breached node yields ciphertext + one key share. Neither is useful alone.*

WHY IT WORKS

Secondary System = Strategic Advantage

WHAT WE CAN DO

- Shut down on suspicion
- Zero clinical impact
- Prioritize protection over availability

WHAT PRIMARY EMRS CAN'T

- Can't shut down during attack
- SLA burden locks them in
- Clinical operations at risk

> *Our downtime costs nothing. A HIPAA breach costs hundreds of millions.*

DIFFERENTIATION

The Exfiltration Gap

mTLS auth independent of Active Directory. BTC-inspired multisig key management. Exfiltration protection no competitor offers.

EXFILTRATION PROTECTION

Rubrik Cohesity / Veeam	EMRrescue	Epic CRO	HIEs	True North Continuity
No	No	No	No	Yes ($$$)
Recovery only	Continuity only	AD-dependent	Not a backup	Recovery + Continuity + Exfiltration

Recovery ✓ Continuity ✓ Exfiltration ✗ ✓ ✓ ✓

Every competitor solves recovery. No one solves theft.

MARKET

$13.7B Market, One Unsolved Gap

Disaster recovery and backup market growing at 12% annually. HIPAA Safe Harbor law (HR 7898) creates a regulatory tailwind — recognized security practices reduce penalties after a breach. Data portability as a secondary wedge expands the addressable market beyond healthcare.

Traction: We have a warm, friendly contact — a CMIO at a major 30-hospital, $6B+ health system who lived through a catastrophic ransomware attack in 2023. A second contact — a CIO at a single hospital within a wider system — experienced a similar attack in early 2025. These are just our immediate, relatively small circles.

THE PRODUCT

Visible Trust

Live verification display: real-time ADT archiving, click-to-reconstruct, audit-logged inspections. Not a dashboard — a productization of trust.

STATUS

Staged De-Risking

1
Pilot network stood up

2
Secondary EMR continuity in simulated outage

3
Key-splitting validated in production

BUILT
Relay network architecture and ADT ingestion capability.

PLANNED
SSS key-splitting, multisig governance, verification UX.

RISK ACKNOWLEDGED
SSS is a design requirement, not yet in production. The cryptography is well-understood (1979). We are raising to fund the implementation.

THE ASK

First Deployment

Funding for production SSS, pilot infrastructure, and clinical validation.

Staged de-risking, not a monolithic build.

This presentation contains forward-looking statements about planned features and capabilities. Actual results may differ. Nothing herein constitutes a binding commitment.